NCCP 00551 · VERSION 7A

Your treatment, explained

MSI-H/dMMR metastatic colorectal cancer after fluoropyrimidine-based combination chemotherapy · NCCP 00551 v7a · Document revised 6 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Nivolumab + ipilimumab · colorectal after chemotherapy

21-day cycle | Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology immediately for new breathlessness, persistent diarrhoea, severe tummy pain, yellow skin or eyes, much less urine, spreading or blistering rash, severe headache, marked weakness, confusion, unusual thirst or passing much more urine. Severe breathing difficulty, chest pain, collapse or seizure needs emergency care. Effects can begin after treatment ends.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment and urgent-contact card is supplied separately and in the wallet-card section of the complete handbook.

Sources: hse · nccp · smpc

Reference contents

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

How each drug works
Treatment stages
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Baseline and cycle-specific checks
Whole treatment, scans and checks
Tablets, visits and practical planning
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Urgent symptoms and care after treatment
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UNDERSTAND YOUR TREATMENT

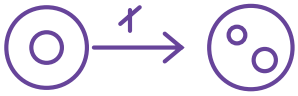
Three medicines. Different jobs.

These two immunotherapy medicines aim to control bowel cancer with a confirmed MSI-H/dMMR DNA-repair feature. This pathway follows previous fluoropyrimidine-based combination chemotherapy.

IMMUNOTHERAPY

Nivolumab

Opdivo



Blocks PD-1

Nivolumab blocks PD-1, a brake on immune T cells. This can help immune cells attack cancer, but can also inflame healthy organs.

Immune T cell → PD-1 brake blocked → Immune attack

What to watch for

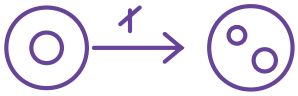
Report new breathing, bowel, hormone, skin, heart or nerve symptoms promptly.

Sources: smpc

IMMUNOTHERAPY

Ipilimumab

Yervoy



Blocks CTLA-4

Ipilimumab blocks another immune brake, CTLA-4, helping activate T cells. Combining two immune medicines can increase anti-cancer activity and also the risk of serious immune inflammation.

Immune activation → CTLA-4 brake blocked → T-cell response

What to watch for

Report diarrhoea, abdominal pain, skin changes, headache, weakness or other new symptoms immediately.

Sources: yervoy

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The anatomical side-effect map follows.

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Hormones / nervous system

Nivolumab + ipilimumab

New severe headache, dizziness, confusion, weakness, thirst or extra urine needs urgent advice; hormone effects may need long-term treatment.

3 Heart

Nivolumab + ipilimumab

Chest pain, racing heartbeat or fainting needs urgent assessment.

5 Kidneys

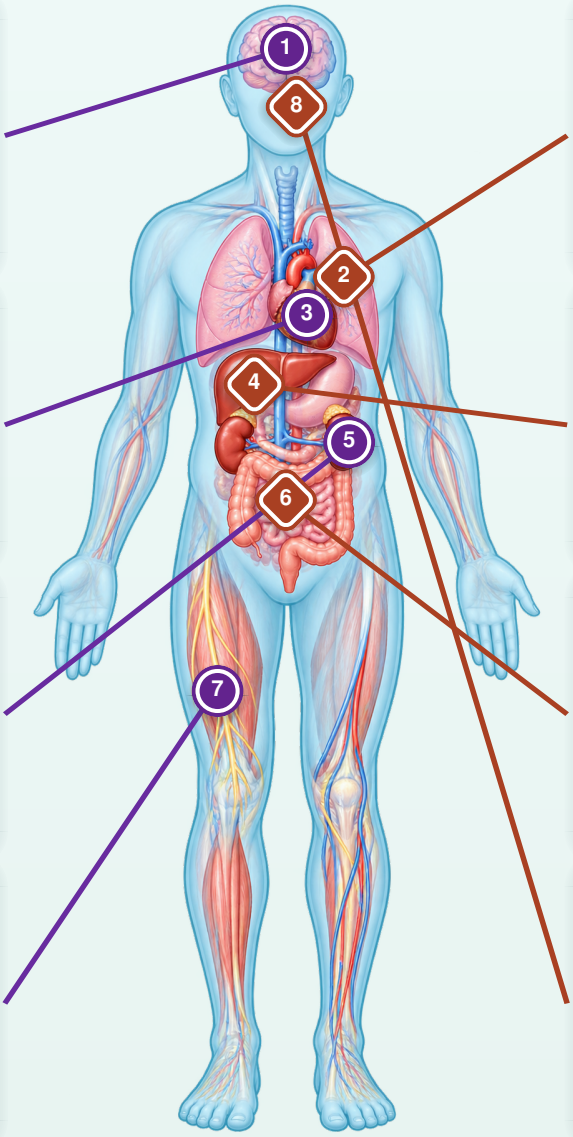
Nivolumab + ipilimumab

Report much less urine, new swelling or a change in kidney tests.

7 Skin / muscles

Nivolumab + ipilimumab

Spreading rash, blisters, severe weakness or muscle pain needs prompt review.



2 Lungs

Nivolumab + ipilimumab

New cough or breathlessness can signal inflammation or another urgent cause.

4 Liver

Nivolumab + ipilimumab

Yellow eyes/skin or dark urine may signal inflammation; tests also check the liver.

6 Bowel

Nivolumab + ipilimumab

New or persistent diarrhoea, blood in stools or severe tummy pain needs urgent advice.

8 Eyes / vision

Nivolumab + ipilimumab

Report eye pain or new visual change promptly; do not wait for the next cycle.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

Treatment → review → next steps

EVERY 21 DAYS

Cycles 1–4: two immunotherapies

C1 C2 C3 C4

Nivolumab 3 mg/kg followed by ipilimumab 1 mg/kg on Day 1. Both are given as infusions. The first nivolumab-only dose is due three weeks after the final combination dose, subject to clinical review.

TEAM-SELECTED MAINTENANCE SCHEDULE

Cycle 5 onwards: nivolumab alone

C5 C6 C7 ...

Nivolumab alone: 240 mg IV or 600 mg under the skin every two weeks, or 1200 mg under the skin every four weeks. Continue while helping and tolerable, until progression or unacceptable toxicity. This colorectal regimen does not set a fixed overall two-year limit.

Your team confirms each stage and discusses any changes with you.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Testing across both treatment phases

Your team books tests and reviews their results. Imaging and tumour-marker timing is individualised; no universal interval is mandated here.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00551 v7a official regimen

THE WHOLE TREATMENT JOURNEY

Nivolumab + ipilimumab · colorectal after chemotherapy: treatment and checks

MSI-H/dMMR metastatic colorectal cancer after fluoropyrimidine-based combination chemotherapy

Cycles 1–4: nivolumab 3 mg/kg, then ipilimumab 1 mg/kg, on Day 1 every 21 days. The first nivolumab-only dose is due three weeks after the final combination dose, subject to clinical review. Nivolumab alone: 240 mg IV or 600 mg under the skin every two weeks, or 1200 mg under the skin every four weeks.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Combination cycles 1–4	Transition to nivolumab alone	Maintenance	After treatment
Blood count, kidneys, liver, glucose and thyroid NCCP 00551	Baseline results	Before every cycle	Review results and symptoms before changing phase	Before every maintenance cycle	Follow persistent abnormalities
Symptoms and other hormones NCCP 00551	Discuss autoimmune disease, infections, medicines and earlier immune treatment	Assess immune symptoms; cortisol if indicated	A phase change does not remove immune risks	Report symptoms immediately; extra tests as indicated	Monitoring for at least five months after the last dose
Treatment and observation NCCP 00551	Confirm MSI-H/dMMR result and correct treatment line	Nivolumab first; then ipilimumab; vital signs and post-infusion observation	First maintenance dose three weeks after final combination	Confirm selected dose, route and two-/four-week interval	Retain the immunotherapy alert card
Scans and response NCCP 00551	Agree assessment plan	Scans and markers as directed	Review benefit and suitability for maintenance	Review benefit, tolerability and duration	Agree next care and follow-up

What each check is for

1

Cycles 1–4: two immunotherapies

Every 21 days

These two immunotherapy medicines aim to control bowel cancer with a confirmed MSI-H/dMMR DNA-repair feature. This pathway follows previous fluoropyrimidine-based combination chemotherapy.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00551

2

Cycle 5 onwards: nivolumab alone

Team-selected maintenance schedule

These two immunotherapy medicines aim to control bowel cancer with a confirmed MSI-H/dMMR DNA-repair feature. This pathway follows previous fluoropyrimidine-based combination chemotherapy.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00551

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.35

YOUR CYCLE, DAY BY DAY

Your 21-day cycle.

Read the timing and symptom notes below. These are possible patterns, not a promise about when symptoms start or stop.

Treatment timing: displayed days 1–21

Treatment or check	Days in this window	What happens
Combination treatment	Day 1	Day 1 in cycles 1–4: nivolumab then ipilimumab
Immune effects	Days 1–21	Any day; also during maintenance or after stopping
Symptoms and tests	Days 1–21	Blood and thyroid tests before each treatment cycle

This 21-day calendar describes the first four combination cycles. Maintenance uses the separately confirmed two- or four-week schedule.

The next cycle starts 21 days after this one, if your team confirms it.

> Immune effects can happen on any day and after treatment ends. Carry your immunotherapy alert card and tell every healthcare team that you have received nivolumab and ipilimumab.

Complete timing and symptom notes

Day 1: Combination visit: cycles 1–4

Blood results and symptoms are reviewed. Nivolumab is given first, then ipilimumab. The team monitors you during treatment and afterwards. Tell the nurse immediately if you become itchy, flushed, breathless, dizzy or suddenly unwell.

Day 2–21: Between combination visits

Immune inflammation can begin at any time; the calendar cannot predict a safe symptom-free week. After cycle 4, confirm the nivolumab-only dose, route, interval and first maintenance date.

Sources: nccp

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Combination appointments

During the first four cycles, nivolumab is infused first over about 30 minutes, then ipilimumab over about 30 minutes. Allow extra time for checks and observation. Nivolumab alone: 240 mg IV or 600 mg under the skin every two weeks, or 1200 mg under the skin every four weeks.

STEP 2

Changing phase

The first nivolumab-only dose is due three weeks after the final combination dose, subject to clinical review. Nivolumab alone: 240 mg IV or 600 mg under the skin every two weeks, or 1200 mg under the skin every four weeks.

STEP 3

Tests and reviews

Before starting and before every cycle: blood count, kidney and liver profile, glucose and thyroid function. Cortisol is checked if clinically indicated. Disease assessment follows the consultant plan. Continue monitoring for late immune effects for at least five months after the final dose.

STEP 4

Alert card and home care

There is no take-home chemotherapy pump. Keep the card, medicine list, symptom record and urgent contact numbers accessible.

STEP 5

Treatment duration

Continue while helping and tolerable, until progression or unacceptable toxicity. This colorectal regimen does not set a fixed overall two-year limit.

Sources: NCCP 00551 v7a official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Sickness prevention and premedication

Nivolumab has minimal and ipilimumab low emetogenic potential. Routine premedication is not usually required; the team individualises anti-sickness treatment and care after any previous reaction.

2 Steroids and other treatment for immune effects

Do not self-start or stop steroids. If prescribed, follow the exact taper and any infection-prevention plan. Do not restart cancer treatment until the team explicitly authorises it. If the team changes the interval between nivolumab-only doses, they will give you a new appointment plan: the first four-weekly dose is two weeks after the last two-weekly dose, or the first two-weekly dose is four weeks after the last four-weekly dose. Do not change the schedule yourself. This does not change which formulations are permitted for your colorectal treatment.

3 Pregnancy and feeding

Discuss fertility and effective contraception before treatment. Effective contraception continues for at least five months after the last nivolumab dose. Do not breastfeed during treatment; agree any later feeding plan with the team.

Sources: nccp · smpc

TREATMENT REFERENCE · 01

Your cancer and treatment aim

A validated tumour result helps select this treatment.

MSI-H and dMMR

These describe a feature of the cancer's DNA-repair system. The result helps identify colorectal cancers that may respond to immunotherapy. Ask whether a genetics assessment is relevant.

Sources: nccp · smpc

Your treatment setting

These two immunotherapy medicines aim to control bowel cancer with a confirmed MSI-H/dMMR DNA-repair feature. This pathway follows previous fluoropyrimidine-based combination chemotherapy.

Sources: nccp · smpc

Choices and goals

Discuss the likely benefit, uncertainty, other treatment options, clinical trials and symptom-focused care in relation to your priorities.

Sources: nccp · smpc

TREATMENT REFERENCE · 02

How the two medicines work

They act at two different immune-control points.

Nivolumab

Nivolumab blocks PD-1, a brake on immune T cells. This can help immune cells attack cancer, but can also inflame healthy organs.

Sources: nccp · smpc

Ipilimumab

Ipilimumab blocks another immune brake, CTLA-4, helping activate T cells. Combining two immune medicines can increase anti-cancer activity and also the risk of serious immune inflammation.

Sources: yervoy

Why side effects need prompt assessment

Immune activity may inflame healthy organs. The combination can cause serious effects, and symptoms may continue or begin after ipilimumab or all treatment stops. Pauses, steroids or permanent discontinuation may be needed.

Sources: nccp · smpc

TREATMENT REFERENCE · 03

Combination first, then nivolumab alone

Keep both phases in your written plan.

Four combination cycles

Nivolumab 3 mg/kg and ipilimumab 1 mg/kg are given on Day 1 every 21 days for four cycles. Nivolumab is infused first, then ipilimumab; each infusion takes about 30 minutes, with additional preparation and observation.

Sources: nccp · smpc

First nivolumab-only treatment

The first nivolumab-only dose is due three weeks after the final combination dose, subject to clinical review.

Sources: nccp · smpc

Maintenance options

Nivolumab alone: 240 mg IV or 600 mg under the skin every two weeks, or 1200 mg under the skin every four weeks.

Sources: nccp · smpc

How long treatment continues

Continue while helping and tolerable, until progression or unacceptable toxicity. This colorectal regimen does not set a fixed overall two-year limit.

Sources: nccp · smpc

Under-the-skin maintenance

The team may select the specific nivolumab formulation given under the skin over about 3–5 minutes. Tell staff about sore, red or bruised skin at the injection site. Dose and route must match the prescription; they are not interchangeable by simple volume.

Sources: nccp

Changing maintenance appointments

If the team changes the interval between nivolumab-only doses, they will give you a new appointment plan: the first four-weekly dose is two weeks after the last two-weekly dose, or the first two-weekly dose is four weeks after the last four-weekly dose. Do not change the schedule yourself. This does not change which formulations are permitted for your colorectal treatment.

Sources: nccp

TREATMENT REFERENCE · 04

Tests and treatment reviews

Checks continue in both phases.

Blood and thyroid tests

Before starting and before every cycle: blood count, kidney and liver profile, glucose and thyroid function. Cortisol is checked if clinically indicated. Disease assessment follows the consultant plan. Continue monitoring for late immune effects for at least five months after the final dose.

Sources: nccp · smpc

Scans and markers

The supervising consultant determines imaging, tumour-marker and other disease-monitoring tests. Ask when each test is due and who will review the result.

Sources: nccp · smpc

Treatment delays

If a dose is held, the team decides whether and when either medicine can restart. Do not assume one medicine is safe to receive when the other has been withheld.

Sources: nccp · smpc

TREATMENT REFERENCE · 05

Between visits and practical support

Keep an early-reporting plan.

Bowels and hydration

Report new diarrhoea, blood in stools or abdominal pain immediately. Immune bowel inflammation can worsen and needs assessment; do not rely on self-treatment alone.

Sources: nccp · smpc

Energy, hormones and daily life

New marked fatigue, headache, dizziness, weakness, thirst or extra urine can need hormone/glucose tests. Ask for help with eating, activity, sleep, work and caring responsibilities.

Sources: nccp · smpc

Medicine list and steroids

Tell the team about all medicines, supplements and steroid use. Follow any steroid taper exactly and carry information about hormone replacement.

Sources: nccp · smpc

Pregnancy and feeding

Discuss fertility and effective contraception before treatment. Effective contraception continues for at least five months after the last nivolumab dose. Do not breastfeed during treatment; agree any later feeding plan with the team.

Sources: nccp · smpc · yervoy

TREATMENT REFERENCE · 06

Urgent symptoms and care after treatment

Do not wait for the next appointment.

What to report

Contact oncology immediately for new breathlessness, persistent diarrhoea, severe tummy pain, yellow skin or eyes, much less urine, spreading or blistering rash, severe headache, marked weakness, confusion, unusual thirst or passing much more urine. Severe breathing difficulty, chest pain, collapse or seizure needs emergency care. Effects can begin after treatment ends.

Sources: nccp · smpc

During infusions

Alert the nurse immediately if you feel itchy, flushed, breathless, dizzy, tight-chested or suddenly unwell.

Sources: nccp · smpc

Late effects

Keep the alert card and report symptoms after treatment stops. NCCP specifies continuous monitoring for at least five months after the last dose. Some hormone changes need longer follow-up.

Sources: nccp · smpc

Evidence for this colorectal setting

CheckMate 142 followed 119 previously treated people receiving nivolumab 3 mg/kg plus ipilimumab 1 mg/kg, followed by nivolumab alone.

The four-year follow-up reported sustained responses in this single-arm cohort. Without a randomised control group, it cannot establish how much longer a person lives compared with another treatment.

Sources: trial · nccp

PREPARE FOR THE CONVERSATION

Questions worth asking

Which colorectal setting and MSI-H/dMMR result apply to me?

What benefit and risks apply to this combination?

When are my four combination cycles and first maintenance visit?

Which maintenance dose, route and interval have been prescribed?

Who checks my blood, thyroid and scan results?

Who do I contact urgently during treatment or after it ends?

> **Try explaining the plan back:** Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00551 v7a. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00551 v7a official regimen
- EMA Opdivo product information and patient leaflet
- EMA Yervoy product information and patient leaflet
- CheckMate 142 four-year combination cohort follow-up
- HSE treatment safety and urgent advice

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-05.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6385/551_Nivolumab_3mgkg_Ipilimumab_1mgkg.pdf

https://www.ema.europa.eu/en/documents/product-information/opdivo-epar-product-information_en.pdf

https://www.ema.europa.eu/en/documents/product-information/yervoy-epar-product-information_en.pdf

https://pubmed.ncbi.nlm.nih.gov/35764271/

https://www2.hse.ie/conditions/chemotherapy/side-effects/